Page of Contents:

Sl.Order	Particulars
A	Purpose
B	Scope
C	Responsibility
D	Departmental Hierarchy
E	Policy
F	Areas of the service 1. For older children 2. Clinical care 3. Inpatient Services 4. Basic guidelines for admitting the patients 5. Patient Referral 6. Emergency Services

©Hope Hospital

A. Purpose: To provide guideline instruction for efficacy in the operational aspect of the department.

B. Scope : Paediatrics department.

C. Responsibility Person: Consultant Paediatrics.

D. Departmental Hierarchy:

Consultant – Paediatrics

↓

Staff Nurse

↓

Housekeeping Staff.

The Pediatrics at Hope Hospital provides comprehensive care for your child from childhood to adolescence. The department comprises of a team of well experienced & qualified Pediatricians and provides the following services:

- Pediatric clinics
- In-patient services:

❖ Neonates are not treated routinely in our organisation. Emergency resuscitation may be performed in a critical neonate and later transferred to super speciality center specialising in neonatology.

E. Policy :

1. **<u>Outdoor Clinics</u>** **:** The paediatric outpatient clinic runs for six days in the week from 8.00 am to 10.00 am.

2. **Emergency Services** : The Emergency services for paediatric patients are not available. As the hospital does not have a dedicated neonatal or paediatric ICU hence such patients are immediately transferred to Colours Hospital where facility for Neonatal and paediatric Intensive Care unit is available.

3. **Inpatient Services** : The inpatient services are meant for patients who require regular monitoring in the inpatient care facilities of the hospital.

4. **Care Person** : Clinical care is provided by post graduate qualified and experienced pediatricians. Nursing care is provided by qualified, trained and registered nursing staffs.

F. The department provides specialized care in the following areas:

1. For older children:

a. Growth and Development Abnormalities
b. Endocrine problems like hypothyroidism
c. Hematological problems like anaemia
d. Renal problems like nephritic syndrome and urinary infections
e. Neurological problems like epilepsy
f. Respiratory problems like asthma and pneumonia
g. Infective problems like typhoid ,dengue and malaria etc.

5. **Clinical Care:**

i. Outpatient Clinic

4. I.i Initial Assessment:

a. As soon as a patient enters the OPD we observe the following:-

A) Look of the child - a) Normal or b) sick looking in appearance

B) General appearance / Behavior :-

i. Consciousness
i.a. dull & lethargic
i.b normal
i.c overactive or imitable

C) Is there any puffiness or pallor over the face

D) Gait
a) Walking normally
b) Alaxia etc

E) Nutritional status of the child

F) Immunization History, etc

G) Growth

The following information is also taken into account as a part of the initial assessment of the child:

- H/O Present illness
- H/O past illness
- Family history.

General examination of the patient includes:-

- Consciousness of the child
- Anemia
- Cyanosis
- Jaundice
- Edema
- clubbing
- nutritional status
- temperature
- P/R
- R/R
- BP – as & when needed
- Lymphadenopathy
- Petechic

With the help of the patient's history & general examination followed by systemic examination, a provisional diagnosis of the patients conditions is made. In light of the above information the relevant investigations are suggested incase needed, so as to ascertain the provisional diagnosis.

However immediate treatment of the patient is initiated on the basis of the provisional diagnosis as there is a time gap in reporting of investigation. Once investigation reports are received the paediatric consultant starts the definitive treatment of the patient. Patient's relatives are explained about the complication and the treatment. They are educated about nutrition, immunization and safe parenting.

5. Inpatient Services:

If the general condition of the patient is poor or critical and requires regular monitoring, the treating consultant immediately admits the patient in the paediatric ward and treat accordingly.

Patients are admitted in the inpatient care facility of the hospital only if the treating paediatric consultant prescribes the same in writing. However in case of emergency the EMO on duty or any other doctor of the hospital may admit the patient in the inpatient care facilities but the final decision regarding continuation of the patient is taken by the paediatric consultant strictly on the basis of the condition of the patient.

6. Basic guidelines for admitting the patients

a. When home care is not adequate for the treatment of the patient.

b. If the patient's condition is poor or critical.

c. When the vitals such as pulse, respirations, B.P etc, are not satisfactory.

d) For any other reason as deemed necessary by the treating doctor in accordance with the condition of the patient.

Prior to the admission of the patient, the condition of the child along with the reasons for his admission is explained clearly to the parents by the treating doctor. Once the decision for admitting the patient is made and consent taken from the relative, the indoor admission file prepared .The treating doctor immediately records the details about the patient and his condition in the indoor files, fills up the investigations form and prescribe the treatment.

The patient is shifted to the paediatric ward along with the ward boy. The sister in-charge of the paediatric ward receives the patient ,allots a bed and makes the patient comfortable, the samples are taken for investigation as prescribed in the indoor file of the patient and treatment is started within 5 to 10 minutes.

However if the patient is in critical state , all the formalities are left behind and treatment is started immediately.

Incase needed, the treating paediatric consultant discuss the patient's condition with the other paediatric consultants of the department or with consultants from other specialties like surgery, gynaecology etc as deemed necessary by the treating consultant.
All the details regarding the treatment of the patients, investigations suggested and their findings , medicines and diet prescribed , patient's vitals at periodic intervals ,progress made etc are recorded in the inpatient file of the patient by the treating doctor and the same is signed, dated and timed.

7. Patients Referral:

The treating consultant may refer the patient to "Colours Hospital" if –

a) The child is not improving or deteriorating in spite of the best effort by the treating consultant.
b) The treating consultant has any about the diagnosis.
c) If further investigations are needed for the treatment of the child which are not available in the hospital.
d) Facility for neonatal or paediatric intensive care are needed for the treatment of the child which are not available in our hospital.

For any other reason as deemed necessary by the treating consultant.

Prior to the transfer of the patient , the treating consultant completes the referral form with the patient's name ,age ,sex ,address ,date and time of admission ,date and time of referral, reasons for the same along with the diagnosis ,investigations done if any and the treatment given.
The treating consultant informs the same over phone to the paediatric consultants of "King George Medical College" and briefs him/her about the patient conditions, treatment given etc and ensures that all the arrangement for the treatment of the patient are made prior to the patient's arrival at the other facility so that immediate treatment of the patient can be initiated.

8. Emergency Services:

i. If the patient arrives in the emergency department of the hospital the Medical Officer on duty attends the patients and the paediatric consultant is informed who immediately attends the patient and initiates further treatment.

ii. If the patient arrives in the hospital after 2.00pm, the emergency medical officer on duty attends the patients and provides the initial treatment .The paediatrician on call is informed over phone who incase needed (depending upon the condition of the patient) attends the patient within 20 minutes.